

Caldwell B. Esselstyn, Jr., MD

PREVENT AND REVERSE HEART DISEASE



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PLANT-BASED NUTRITION

by Caldwell B. Esselstyn, Jr., MD

The following remarks by Caldwell B. Esselstyn, Jr., MD were delivered September 2, 2000 in Orlando, Florida at the Summit Conference on Cholesterol and Coronary Risk presented by the Cleveland Clinic Foundation in cooperation with the Walt Disney World Company.

These comments are a reflection of epidemiologic survey information and Dr. Esselstyn's 12-year reversal study of patients with severe coronary disease. In the longest study of its type, the author has demonstrated elimination of disease progression in all compliant patients who maintained a total serum cholesterol less than 150 mg/dl and an LDL cholesterol less than 80 mg/dl through a plant-based diet and cholesterol-reducing medications.

PLANT-BASED NUTRITION

NEW!

Read Dr. Esselstyn's article from *The American Journal of Cardiology*, entitled:

[*Is the Present*](#)

[*Therapy for Coronary Artery Disease the Radical Mastectomy of the Twenty-First Century?*](#)

(pdf file)

Plant-based nutrition provides us with a pathway to escape the coronary artery disease epidemic. For persons in central Africa, the Papua Highlanders of New Guinea, the Tarahumara Indians of northern Mexico, and inhabitants of rural China as described in the Cornell China Study, coronary disease is essentially non-existent while hypertension, Western malignancies, obesity, and adult onset diabetes are rarely encountered.

While I am proud to have been the founder of this conference in 1997, it has become ever more apparent that these conferences are not the ultimate answer. This stopgap risk factor and drug oriented device-driven approach is not designed to conquer this epidemic. This strategy is laden with expense, morbidity, mortality, and temporary benefits which rapidly erode with time. We must focus on the toxic food environment for otherwise our children and young adults will become the next unsuspecting victims.

We have a crises of leadership in our public and private institutions with an emphasis on prevention. Their advice to the public of 30% fat in the diet guarantees

disease development and progression. This level advocated by the National Research Council, the American Heart Association, the National Cholesterol Education Program, and the National Institutes of Health has been shown scientifically to worsen the disease. For them to sanctify this diet as healthy for the American public, is egregiously inaccurate. Like trying to fit Cinderella's slipper on one of her sister's - it simply doesn't work. By way of contrast, the American Cancer Society prefers 20% dietary fat, while the World Health Organization advocates 15%.

In 1987 Dr. Scott Grundy proclaimed that with a blood pressure of 110/70 and a total cholesterol of under 150 mg/dl, 90% of heart attacks could be avoided. Sadly, no public or private institution detailed for the public the plant-based diet that could achieve those goals.

The United States Department of Agriculture has a sorrowful record of caving to the special interests of industry as does the U.S. Congress. The U.S. Dietary Guidelines committee has over half its members with ties to industry, while the chairman of the health and human services department wears a milk mustache. For those institutions to decide America's nutritive needs is surely putting the fox in the hen house.

A slogan has arisen from these groups stating there are no strictly good foods or bad foods. This is sheer nonsense. Coronary disease has no building blocks from the plant-based diet. Meat, cheese, milk, butter, ice cream, eggs, fried foods, oils, and margarine are the lethal atherosclerotic lynchpins. Vogel demonstrated endothelial damage in young persons within hours of eating such foods. English children under 10 years of age have been found by ultrasound to be losing arterial elasticity and distensibility. The Bogalusa, PDAY, Korean, and Vietnam data reveal coronary artery disease is ubiquitous in our young.

We require a new yardstick to measure cardiovascular excellence in our institutions. No longer will the number or quality of interventions and their temporary benefits suffice. There is ample evidence-based research to support efforts geared to prevention. We must ask how many patients have achieved successful arrest and reversal and avoided new coronary and cardiovascular events through preventive lifestyle changes? Some will argue that their patients might not follow a plant-based diet. This, I increasingly find, is a totally negative and self-serving speculation. Our experience is that patients rejoice that they are now empowered to abolish their disease progression and are often furious that they were not made aware of this option earlier. They clearly recognize that the locus of control for this disease is vested with them. While this conference will continue updating us with mechanisms of disease, that information alone will never shut down atherosclerotic development.

The truth be known, we don't need new information to end this epidemic. The evidence is in! As practicing physicians, dietitians, nutritionists, nurses, and researchers, you are an essential part of the vanguard to disseminate the facts to patients, the public, and the government.

As leaders, we must have the courage and a renewed moral compass to provide the public with accurate information to abolish and prevent atherosclerosis. We must seek leaders and institutions that are not frayed or compromised by ties to industry or politics to deliver this message. It must be science and not the messenger which dictates public policy.

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